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Defining social prescribing link worker practice in Europe: an international Delphi consensus study of core functions, training needs and implementation conditions

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ABSTRACT

Objective To develop expert consensus on best practice approaches to social prescribing (SP) in European primary care, with a focus on the roles, competencies and implementation conditions for social prescribing link workers (SPLWs).

Design A three-round Delphi study conducted online. Round 1 used open-ended questions to map current practice and inform statement development; Round 2 rated 52 statements on a 5-point Likert scale with an a priori approval threshold of ≥80% agreement (agree/strongly agree); Round 3 rerated 19 statements using approval (≥80% agree/strongly agree) and rejection (≥80% disagree/strongly disagree) thresholds.

Setting Virtual (online survey platform).

Participants An international, multidisciplinary expert panel recruited via relevant SP networks and targeted invitations, including researchers, healthcare professionals, link workers and implementation stakeholders in accordance with the published study protocol.

Results Round 1 was completed by 75/85 invited panellists (88%). In Round 2, 57/75 (76%) rated 52 statements across six domains; 19/52 met the ≥80% agreement threshold, with strongest consensus for core SPLW functions (navigation, advocacy, follow-up and documentation) and for support needs (mandatory communication/navigation training and supervision). Round 3 was completed by 57/57 (100%) and approved 13/19 rerated statements, extending consensus to include data collection with appropriate safeguards, optional accompaniment to community activities, limited first-step support for urgent social problems, context-sensitive funding principles, a cofunded employment model, multiple access routes (including telephone/in-person/online), self-referral and secondary-care referral, and the use of digital tools as complements rather than substitutes for relational support.

Conclusion Across three Delphi rounds, the panel converged on actionable, consensus-based recommendations for SPLW practice and for accessible, context-adaptable implementation.

STRENGTHS AND LIMITATIONS OF THIS STUDY

⇒ Combines an exploratory Round 1 (open-text mapping) with Rounds 2 and 3 item-level agreement data across six implementation domains.
⇒ Uses a stringent consensus threshold (≥80% agreement) and structured feedback consistent with Conducting and REporting DElphi Studies guidance.
⇒ This manuscript reports findings from all three Delphi rounds and presents the final consensus-based best practice statements for social prescribing link workers and implementation conditions.
⇒ Participant eligibility was restricted to individuals with English language proficiency (including speaking, reading and writing), which may introduce selection bias and limit the generalisability of the findings to non-English-speaking populations.
⇒ Due to an excessive amount of missing data for the 'years of experience' variable, it was not possible to provide a meaningful or representative description of the panel.

INTRODUCTION

Social prescribing (SP) is an integrative approach that aims to address social determinants of health (SDoH) through non-clinical interventions, often via referral pathways in primary care and linkage to community resources.[1–15] SP has been linked

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with improved well-being, reduced loneliness and more appropriate use of health and social care resources, underscoring its growing relevance as European health systems seek sustainable, person-centred responses to complex psychosocial needs. Despite rapid growth and policy interest across Europe, implementation varies widely between countries and settings, and there remains limited standardisation around the role of social prescribing link workers (SPLWs).[16–27] Link workers can act as connectors between primary care, patients and community services, yet qualitative and mixed-methods literature indicates variability in job titles, scope of practice, training and professional identity, with ongoing calls for clearer role definition and structured support.[16–27] Across Europe, SP has been promoted as a policy lever to strengthen person-centred, community-oriented care by linking patients to voluntary, community and social enterprise (VCSE) resources.[14] The WHO Regional Office for Europe has provided an implementation toolkit emphasising governance, referral pathways, workforce development and evaluation as core building blocks for scaling SP in diverse health-system contexts.[28]

The national roll-out through Primary Care Networks in England has formalised the SPLW role and articulated expectations for supervision, development and safe practice within a structured workforce framework.[29] Professional development pathways, including accredited training standards for the personalised care workforce, are increasingly used to support consistent practice and role sustainability.[30 31] Despite rapid expansion, evidence syntheses continue to highlight heterogeneity in models, outcomes and measurement, limiting comparability and transferability of findings.[32 33] These challenges underscore the need for clear, consensus-based statements describing link worker functions, competencies and boundaries that can be adapted across European settings. The World Organization of National Colleges, Academies and Academic Associations of General Practitioners (WONCA) Europe Special Interest Group on Social Prescribing and Community Orientation (SPCO SIG) acknowledges the transformative potential of SP in facilitating integrated care models. Using specialised expertise in rural health and community engagement, the group aims to provide conceptual clarity regarding SP and the specific functions of SPLWs. Furthermore, the initiative seeks to map existing programmes and delineate best practices for implementation within heterogeneous healthcare landscapes.[34]

This research endeavours to refine the academic and clinical understanding of SP by identifying the fundamental components necessary for its successful integration into modern healthcare systems. Given its capacity to mitigate health disparities, enhance clinical outcomes and foster community resilience, SP represents a significant mechanism for establishing equitable primary care frameworks across Europe. Conducted under the auspices of WONCA Europe, this project is spearheaded by SPCO SIG members in collaboration with the European General Practice Research Network (EGPRN) and is supported by an EGPRN research grant. To support coherent policy, training and evaluation, the published study protocol proposes a Delphi process aimed at establishing best practices for SP in European primary care, with a specific focus on SPLWs.[34] The protocol specifies consensus development across domains including the SPLW role, qualifications and training, funding, employment, access/referral pathways and target populations. This manuscript reports findings from the completed Delphi study.

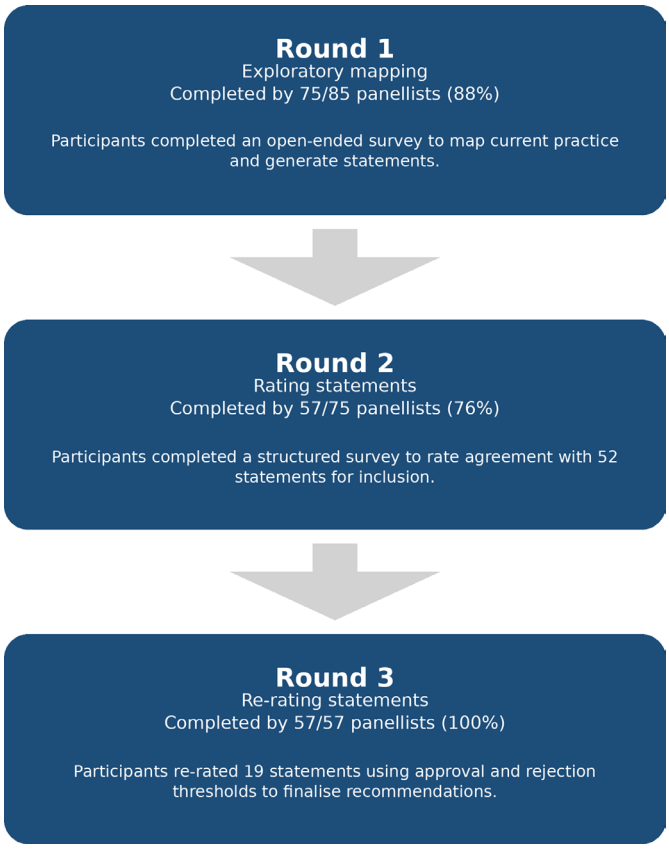


Figure 1 Delphi study flow chart.

METHODS

Study design

A three-round e-Delphi consensus study was conducted to develop best-practice recommendations for SPLWs in European primary care, as specified in the published protocol.[34] An international multidisciplinary panel of experts was involved in this study. Response and retention across the three rounds were: Round 1, 75/85 (88%); Round 2, 57/75 (76%) and Round 3, 57/57 (100%) (see Results and figure 1 for full details).

Setting and survey platform

The study was conducted between April and December 2025. All rounds were administered in English using Welphi (www.welphi.com), an online platform designed for Delphi studies. Before each round, the survey was developed and pilot-tested. Participants received a notification

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email 10 days prior to the launch of each round and, at the start of the round, an email containing the survey link and a calendar invitation with round details. Each round was open for 2 weeks; participants could save their progress, leave and return, and modify their submission until round closure. Surveys took approximately 15 min to complete. Non-responders were removed from subsequent rounds, as prespecified in the protocol.

Panel eligibility and recruitment

Given the absence of a single definition of 'expert' in Delphi research, eligibility criteria were intentionally broad to capture multidisciplinary and stakeholder perspectives. Eligible panellists included: individuals involved in the SPCO SIG (WONCA Europe), the Global Social Prescribing Alliance or the National Academy for Social Prescribing; authors of academic or grey literature related to SP; researchers or healthcare providers engaged in SP activities; link workers facilitating SP (regardless of local terminology) and healthcare administrators or managers overseeing implementation. Recruitment commenced in November 2024 with a registration survey circulated to SPCO SIG members and through targeted invitations. The steering group compiled country-specific lists of potential experts and used publicly available sources to identify contact details. To promote diversity by country, professional background, social prescribing expertise and experience, recruitment was monitored and targeted invitations were additionally disseminated via relevant newsletters/listservs (eg, SP Network; Social Interventions Research and Evaluation Network) and social media (X/Twitter; LinkedIn). Snowball sampling was encouraged. The registration survey collected sociodemographic information (name, email, country, job title, organisation, self-reported SP expertise/experience and age in years). Name, email and organisation were collected solely to enable survey distribution, scheduling and identity verification; these identifiers were not linked to responses in any analysis and were not accessible to the research team once anonymised codes were assigned (see Confidentiality, anonymity and data handling below). Age was additionally collected as a standard sociodemographic descriptor, consistent with common Delphi reporting practice, although it was not explicitly prespecified as a variable in the published protocol; this addition did not affect eligibility, randomisation or any other study procedure and is reported in table 1. Years of SP-related experience were also collected but are not reported due to a high proportion of missing responses for this item (see Limitations).

Table 1 Sociodemographic characteristics of Delphi panel respondents by study round

Characteristic	Round 1	Rounds 2–3
Country		
UK	16 (21.3)	12 (21.1)
Germany	10 (13.3)	8 (14.0)
Italy	7 (9.3)	5 (8.8)
Romania	7 (9.3)	5 (8.8)
Spain	6 (8.0)	5 (8.8)
Czech Republic	5 (6.7)	4 (7.0)
Portugal	4 (5.3)	3 (5.3)
Sweden	4 (5.3)	3 (5.3)
Austria	4 (5.3)	3 (5.3)
Croatia	4 (5.3)	3 (5.3)
Serbia	3 (4.0)	2 (3.5)
France	3 (4.0)	2 (3.5)
Poland	2 (2.7)	2 (3.5)
Gender		
Female	48 (64.0)	36 (63.2)
Male	27 (36.0)	21 (36.8)
Age		
Mean (SD), years	50.6 (11.2)	50.6 (11.2)
Median (IQR), years	50.0 (14.0)	50.0 (14.0)
Range, years	27–75	27–75
Age group		
18–34	5 (6.7)	4 (7.0)
35–44	15 (20.0)	11 (19.3)
45–54	25 (33.3)	19 (33.3)
≥55	28 (37.3)	21 (36.8)
Missing/not reported	2 (2.7)	2 (3.5)
Professional role		
Doctor/physician	33 (44.0)	25 (43.9)
Academic/researcher	16 (21.3)	12 (21.1)
Social worker	8 (10.7)	6 (10.5)
Nurse	5 (6.7)	3 (5.3)
Link worker	4 (5.3)	3 (5.3)
Education/youth work	1 (1.3)	1 (1.8)
Programme/project management	1 (1.3)	1 (1.8)
Consultant/trainer	1 (1.3)	1 (1.8)
Commissioning/policy	1 (1.3)	1 (1.8)
Public health	1 (1.3)	1 (1.8)

Values are n (%), unless otherwise stated. Round 1 respondents n=75; Rounds 2–3 respondents n=57.
Years of experience in social prescribing were collected at registration but are not reported owing to a high proportion of missing responses for this item.

Delphi rounds and procedures

Round 1 → Round 2

Round 1 (April 2025) comprised an open-ended survey to elicit foundational perspectives on the SPLW role, qualifications, skills and training, funding and employment scheme, and target populations expected to benefit most from SP. Round 1 responses were synthesised to generate candidate statements for subsequent rounds. The research team leaders conducted qualitative content analysis of participants' free-text responses. Two senior researchers independently reviewed all responses and developed an initial coding framework through an iterative

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process. First, they conducted open coding on a subset of responses to generate preliminary codes, then compared and consolidated these into a shared codebook with brief definitions and inclusion/exclusion criteria. The consolidated codebook was applied to the full dataset, with codes refined as needed when new concepts emerged. Discrepancies in coding were resolved through discussion; where consensus could not be reached, a third researcher adjudicated. Codes were then grouped into higher-order categories and themes. Themes were primarily inductive (derived from participants' responses) but were also mapped to a priori domains informed by the literature on SP and link worker roles to ensure comprehensive coverage. We monitored for thematic saturation during coding and proceeded to statement drafting once no substantively new themes emerged. Themes were translated into draft Delphi statements by converting thematic summaries into single-issue items. The study team reviewed all items to ensure neutrality, remove duplication, split compound statements where needed, and optimise comprehensibility. This process generated the Round 2 questionnaire, comprising the main statements across different domains.

Round 2 → Round 3

Round 2 (June 2025) presented the synthesised items as a structured questionnaire; panellists rated each statement on a 5-point Likert scale (1=strongly disagree to 5=strongly agree) and could provide free-text comments to suggest refinements. Following Round 2, free-text comments were reviewed by the study team and used to refine the wording of statements that did not reach consensus, to improve clarity and cross-country applicability (eg, clarifying terms, specifying the referent population or separating compound statements into single-issue items; for example, a statement referring broadly to 'the healthcare system' was revised to specify 'primary care services' to reduce ambiguity across differing national health system structures). Revisions were limited to wording changes intended to improve clarity and cross-country applicability; items reflecting substantively different intents were not merged.

Round 3

Round 3 (December 2025) readministered the 19 statements that did not meet the prespecified consensus threshold in Round 2, using the revised wording described above where applicable. Participants rated each statement again using the same 5-point Likert scale.

Round 3 served to finalise recommendations by rerating the non-consensus items from Round 2 using the same approval criterion (≥80% agree/strongly agree) and an additional rejection criterion (≥80% disagree/strongly disagree). This process resulted in a final set of consensus-based statements and identified a smaller group of issues where agreement remained below the threshold, indicating areas requiring local adaptation and further evaluation.

In Round 3, participants received structured feedback (their previous rating and the Round 2 response distribution) and were invited to reconsider and comment before rerating each item. Participants were encouraged to reconsider their rating in light of group feedback and could add explanatory comments. The Round 3 results were then used to determine whether consensus was reached and to identify the final set of recommendations regarding SPLW roles, education/training requirements and target populations.

Confidentiality, anonymity and data handling

To ensure confidentiality and mitigate potential bias, participants were assigned randomly generated codes by the survey platform. Codes were not shared with participants, and the research team had no key linking identities to codes. The anonymised database is retained solely for scientific purposes; the principal investigator is responsible for data storage and handling, and anonymised raw data are available from the corresponding author on reasonable request.

Consensus definition

Consensus thresholds were defined a priori. In Rounds 2–3, 'approval' required ≥80% of panellists selecting agree or strongly agree (ratings 4–5). In Round 3, 'rejection' was additionally defined as ≥80% selecting disagree or strongly disagree (ratings 1–2). Round 1 open-text responses were analysed using qualitative content analysis: two senior researchers independently reviewed and categorised responses to identify key themes, which informed statement development. Round 2 pooled results were summarised descriptively as percentages for each response option; agreement was calculated as the sum of agree and strongly agree. Round 3 results were summarised analogously, reporting both approval and rejection proportions.

Data analysis

The data collected from the registration survey were analysed with IBM SPSS Statistics for Windows V. 29 (IBM Corp, Armonk, USA).[35] Qualitative data were presented for each survey item and analysed through qualitative content analysis. All rounds were administered in English using Welphi (www.welphi.com), an online platform designed for Delphi studies.

Quality and transparency

This study was conducted and reported in accordance with the Conducting and REporting DElphi Studies guidelines (see online supplemental material 1).[36]

Patient and public involvement

Patients and/or the public were not involved in the design, conduct, reporting or dissemination plans of this research.

Data protection

Participation was voluntary. Prior to enrolment, participants were presented with an online information page

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describing the study purpose, procedures, expected time commitment, data handling and confidentiality arrangements, and their right to withdraw at any time without consequence. Informed consent was indicated by proceeding beyond this information page to the survey; this procedure was applied consistently, and consent remained ongoing, across all three rounds.

The principal investigator is responsible for data handling, storage and management.

The anonymised raw data that supported the findings of this study are available from the corresponding author, on reasonable request.

RESULTS

The flow chart of this study is illustrated in figure 1.

Round 1: exploratory mapping

Panellists

A total of 75 respondents completed the Round 1 survey. Overall response and retention across rounds are summarised in figure 1 (Round 1: 75/85; Round 2: 57/75; Round 3: 57/57). Respondents were predominantly female (64.0%), with a mean age of 50.6 years (SD 11.2; range 27–75) (table 1). Participants represented 15 European countries/UK nations, with the largest contributions from Germany (13.3%), the UK (all nations combined: 21.3%), Italy (9.3%) and Romania (9.3%) (table 1). The most frequently reported professional role was physician/general practitioner (GP) (44%), followed by researcher/academic (21.3%) and social worker (10.7%) (table 1).

Link worker availability and terminology

Perceived availability of a link worker (or equivalent role) varied. 41.3% reported that the role exists in their country, whereas 42.7% reported it does not (table 2). A further 8.0% indicated the role exists under a different title/professional function, and 4.0% described pilot or limited implementation (table 2).

Terminology also varied considerably. The most common naming conventions mapped to 'navigator/connector' (28.0%) and 'coordinator/facilitator' (22.7%), while 9.3% used terms aligned with social work/social services roles (table 2).

Perceived competencies and functions of link workers

Across responses, suggested educational preparation most often emphasised a general degree plus role-specific training (53.3%), frequently alongside backgrounds in social work/social care (48.0%) and health/nursing/medicine (44.0%) (table 3). Psychology/counselling preparation was mentioned by 28.0% (table 3).

Regarding core functions, respondents most frequently described link workers as responsible for navigation/referral to community resources (69.3%), provision of personalised support and follow-up (53.3%), and assessment and goal setting (38.7%) (table 3). Cross-sector coordination/liaison was noted by 22.7%, while monitoring/evaluation was less commonly emphasised (8.0%) (table 3).

Table 2 Link worker availability, terminology, employment models, referral pathways and target populations (Round 1)

Characteristic	Respondents (n=75)
Link worker availability	
Yes	31 (41.3)
No	32 (42.7)
Pilot/limited implementation	3 (4.0)
Exists under a different role or title	6 (8.0)
Other/unclear	3 (4.0)
Terminology used for link worker/social prescriber	
Navigator/connector	21 (28.0)
Coordinator/facilitator	17 (22.7)
Social worker/social services	7 (9.3)
Health mediator/community health worker	1 (1.3)
Volunteer/peer supporter	2 (2.7)
Not applicable/no such role	4 (5.3)
Other term (unclassified)	11 (14.7)
Missing	12 (16.0)
Preferred employment model	
Municipality/local government/social services	30 (40.0)
Health system/primary care	16 (21.3)
Third sector/non-governmental organisation/community organisation	5 (6.7)
Mixed/commissioned funding	5 (6.7)
Volunteer/paid per referral	1 (1.3)
Suggested referral pathways	
Primary care/general practitioner	52 (69.3)
Self-referral	38 (50.7)
Multiple pathways (mixed)	29 (38.7)
Community/voluntary sector	14 (18.7)
Secondary care/hospital	12 (16.0)
Social services/municipality	5 (6.7)
Suggested target populations for social prescribing	
Everyone/universal access	43 (57.3)
Mental health/loneliness/well-being	31 (41.3)
People with social needs/social determinants of health-related issues	20 (26.7)
Long-term conditions/multimorbidity	19 (25.3)
Older adults	15 (20.0)
Lifestyle/behaviour change	5 (6.7)
Children/young people	3 (4.0)
High service use/frequent attenders	2 (2.7)

Values are n (%). Percentages may exceed 100% where multiple responses were permitted.

Employment models, referral pathways and target populations

Preferred employment arrangements most commonly positioned link workers within municipal/local

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Table 3 Thematic content of responses on link worker preparation and functions (n=75)

Theme	Respondents, n (%)
Suggested educational/background preparation	
Generic degree+specialist training	40 (53.3)
Social work/social care	36 (48.0)
Health/nursing/medicine	33 (44.0)
Psychology/counselling	21 (28.0)
Lived experience/peer	4 (5.3)
Arts/culture/exercise specialist	3 (4.0)
Suggested core functions of the link worker	
Navigation/referral to community resources	52 (69.3)
Personalised support and follow-up	40 (53.3)
Assessment and goal setting	29 (38.7)
Coordination/liaison across sectors	17 (22.7)
Health promotion/prevention	8 (10.7)
Administrative/management	7 (9.3)
Empowerment and behaviour change	6 (8.0)
Monitoring and evaluation	6 (8.0)

Percentages reflect the proportion of respondents whose open-text responses mentioned each theme; themes are not mutually exclusive, and so total percentage exceeds 100%.
Owing to its length, the full set of Round 2 and Round 3 approval percentages and final consensus decisions for all 52 Delphi statements is provided in online supplemental table S1 rather than in the main text.

government or social services (40.0%), followed by employment within the health system/primary care (21.3%) (table 2).

In terms of access routes, respondents most frequently endorsed primary care/GP referral (69.3%) and self-referral (50.7%); 38.7% explicitly advocated for multiple referral pathways (table 2).

Suggested target groups often reflected a universal or needs-based approach: 57.3% stated SP should be available to everyone/universal access, while many additionally highlighted people experiencing mental health concerns/loneliness (41.3%), social needs/SDoH-related issues (26.7%), long-term conditions/multimorbidity (25.3%) and older age/frailty (20.0%) (table 2).

Round 1 generated qualitative themes on SPLW roles, competencies, training/support, funding/employment arrangements, access routes and target groups. These themes were converted into a structured list of candidate Delphi statements, with duplication removed and wording refined to ensure each item addressed a single issue and was applicable across European settings. The resulting 52 statements were organised into six protocol-defined domains and presented for quantitative rating in Round 2.

Round 2: statement rating and consensus

Round 2 included 52 statements organised into six domains reflecting the protocol's implementation focus: SPLW roles (16 items), qualifications/training (4), funding (8), employment (8), access/referral pathways (11) and target populations (5). Sociodemographic characteristics of respondents (n=57) in Round 2 and Round 3 are illustrated in table 1.

Consensus overview

19 of 52 statements reached the ≥80% consensus threshold. The highest agreement was observed for the statement that SPLWs should explore 'what matters' to the patient and act as a bridge between providers, patients and community resources (100% agreement). The lowest agreement (rejection) concerned whether a well-programmed app could fully replace the role of a link worker (16% agreement).

Domain 1: SPLW roles (16 items)

13 statements reached consensus (81%–100% agreement), supporting a role that includes helping patients navigate services for social needs, advising on community activities, networking and identifying gaps in services, advocacy, care planning, documentation, involving family/caregivers, follow-up, crisis identification, supporting self-sufficiency and providing motivational/emotional support. Three statements did not reach consensus: collecting sociodemographic/epidemiological or patient-reported outcome data, accompanying patients to activities if they wish, and providing basic social counselling to resolve urgent social problems.

Domain 2: qualifications and training (four items)

Two statements reached consensus: mandatory training in empathic communication, active listening, motivational interviewing and community resource navigation, and provision of regular supervision and continuing professional development. Statements proposing a specific professional background or certification/formal programmes to standardise competencies across countries did not reach consensus.

Domains 3–4: funding and employment models (16 items)

No proposed funding or employment arrangement achieved the ≥80% threshold. Agreement levels were generally low-to-moderate across options including national or regional healthcare/social care funding, insurance-based funding, community funding and co-funded models.

Domain 5: access and referral pathways (11 items)

Three statements reached consensus: access via professional referral by primary care providers, by mental health professionals and by social workers. Self-referral and referral via community-based organisations were approached but did not meet consensus, while telephone helplines and use of apps to support link working if resources are scarce did not reach consensus. Replacing

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link workers entirely with an app was rejected. Notably, only 3 of the 11 access/referral statements in this domain reached consensus, indicating limited overall agreement on preferred access routes beyond professional referral.

Domain 6: target populations (five items)

Only one statement reached consensus: all patients belonging to the most vulnerable groups should be eligible for SP regardless of clinical diagnosis. Statements prioritising social isolation, multimorbidity or health inequities, and universal eligibility for those with psychosocial needs did not reach consensus.

Summary of Round 2 findings

In Round 2, statements reaching the predefined approval threshold (≥80% agree/strongly agree) were retained as consensus recommendations. Statements below the 80% approval threshold were reviewed alongside free-text comments to determine whether non-consensus reflected ambiguity (prompting wording revisions) or substantive disagreement (prompting rerating without changes). 19 statements were carried forward to Round 3. This judgement was made jointly by the study team based on the content of free-text comments (eg, requests for clarification vs explicit disagreement with the underlying principle), with disagreements resolved by discussion among the lead investigators (FP, JV-A and TK).

Round 3

Round 3 ratings were summarised using the distribution of responses across a five-point Likert scale (strongly disagree to strongly agree). Consensus for inclusion ('approval') was defined a priori as ≥80% of panellists selecting 'agree' or 'strongly agree' (ratings 4–5). Consensus for exclusion ('rejection') was defined as ≥80% selecting 'disagree' or 'strongly disagree' (ratings 1–2).

Applying these thresholds to the 19 rerated statements, 13 met criteria for approval, two met criteria for rejection and four remained below consensus (see online supplemental table S1).

Approved statements emphasised the potential for a broad link worker role, including referral to community activities (and optional accompaniment), collection of patient-reported or sociodemographic data with appropriate safeguards, and limited first-step support for urgent social problems. Strong consensus was also observed for funding models that are context-sensitive, for maintaining multiple access routes (including telephone, in-person and online), for availability of self-referral and referral from secondary care, and for cautious use of digital tools as complements rather than substitutes for relational support. Finally, panellists agreed that SP should proactively reach groups at risk of social isolation, multimorbidity and health inequities, while not excluding others who may benefit. Six statements were not approved: two met the predefined rejection criterion, and four remained below consensus. These six statements were: link worker professional background (2-1); formal certification/training standardisation (2-3); avoiding diversion of funds from community provision (3-10); pharmacist referral (5-3); physiotherapist/occupational therapist referral (5-4) and referral from community-based organisations (5-7). Three of these were close to the approval threshold (2-1, 2-3 and 5-7), whereas the two statements proposing exclusion of pharmacist and physiotherapist/occupational therapist referral pathways (5-3 and 5-4) met the predefined rejection criterion (≥80% disagree/strongly disagree) (see online supplemental table S1).

DISCUSSION

This study adds an explicitly European, consensus-based description of SPLW core functions and implementation conditions that can be used to inform role descriptions, training curricula and evaluation frameworks across heterogeneous health-system contexts.[34]

Round 1 highlighted substantial heterogeneity across European settings in whether an SPLW (or equivalent) role exists, the terminology used and the organisational location of link work. Despite this variation, respondents described a broadly consistent set of core functions centred on navigation to community resources, personalised support and follow-up, and assessment/goal setting. These Round 1 findings informed the development of structured statements for Round 2.

Round 2 showed strong convergence on what SPLWs should do in day-to-day practice, particularly person-centred assessment ('what matters'), navigation and advocacy functions, structured follow-up and documentation, alongside clear support needs for mandatory communication/community-navigation training and ongoing supervision/continuing professional development.

Round 3 consolidated and extended this consensus. 13 of 19 rerated statements were approved, including that link workers may collect patient-reported and sociodemographic data with appropriate consent and data-protection safeguards, may accompany people to community activities if desired, and should be competent to offer limited first-step support for urgent and straightforward social problems where immediate guidance or referral is needed. At the system level, panellists approved the principle that funding must take national and local contexts into account and endorsed a cofunded, multisector employment model as a sustainable approach. Lower consensus on funding and employment models may reflect structural differences between European primary care and social care systems—for example, the contrast between predominantly tax-funded universal health systems and mixed public–private or insurance-based models—and the current heterogeneity of SP implementation models.[33] More broadly, non-consensus items after Round 3 clustered in the funding/employment and access/referral-restriction domains, indicating that structural, system-level questions remain less

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amenable to a single European-wide standard than core SPLW functions.

Access and equity considerations also strengthened in Round 3. Panellists supported maintaining multiple access routes (telephone, in-person and online), availability of self-referral and referral from secondary care with clear boundaries, and agreed that SP should proactively reach people at risk of social isolation, multimorbidity and health inequities without excluding others who may benefit. Together with the sustained rejection of replacing link workers entirely with an app, this reinforces the importance of relational, trust-based support as a defining feature of link working.

Several areas remained without consensus after Round 3, including whether SPLWs should have a specified professional background, whether formal certification should be standardised across countries, and whether referrals from particular professional groups or community-based organisations should be restricted. These unresolved items likely reflect legitimate variation in European primary care and social care systems and point to priorities for future comparative implementation research. Future studies could examine how local workforce availability, governance arrangements and VCSE capacity shape the feasibility and outcomes of these design choices.

In this three-round European Delphi study (Round 1: 75/85; Round 2: 57/75; Round 3: 57/57), the panel reached consensus on a comprehensive set of best practice recommendations for SPLWs in primary care. Consensus was strongest for core, person-centred link worker functions and for structured support (training, supervision and continuing professional development).

Several approved statements mirror elements of existing national competency and workforce frameworks, particularly around person-centred assessment, coproduced goal setting, safeguarding, and the need for regular supervision and reflective practice.[30–32] This convergence strengthens confidence that the agreed statements reflect practical requirements for safe and effective delivery, while our findings extend these frameworks by foregrounding equity-oriented access routes and cross-sector relationship building that may be especially salient in heterogeneous European systems.

From a methods perspective, our three-round e-Delphi approach aligns with emerging consensus-reporting guidance.[37–43] The ACCORD reporting guideline and the DELPHISTAR recommendations both emphasise transparent description of panel composition, prespecified consensus criteria, feedback procedures between rounds and reporting of how statements were revised across iterations.[44 45] By documenting panel composition, prespecified consensus thresholds, response rates across rounds and full round-specific voting, this study addresses key elements of these reporting recommendations.

The accelerating pace of SP implementation heightens the importance of monitoring unintended consequences, including variable uptake across sociodemographic groups and potential widening of inequalities.[2] Maintaining multiple access routes, culturally competent practice and strong VCSE partnerships—areas that strengthened in Round 3—may help mitigate these risks.[7] Future evaluations should combine implementation and outcome measures and use more standardised approaches to referral pathway description and outcome selection to support synthesis across countries and models.

Round 3 further endorsed context-adaptable principles for implementation. These included tailoring funding to national/local systems, sustaining multisector employment solutions, maintaining multiple access routes (including self-referral and secondary-care referral) and using digital tools only to complement, rather than replace, relational support. Given potential digital exclusion, implementation should monitor uptake across sociodemographic groups to avoid widening inequalities when digital tools are introduced. Remaining non-consensus items highlight where local adaptation and further evaluation will be needed when scaling SP across diverse European contexts.

A key limitation of this study is that participation was restricted to individuals able to speak, read and write English. This may have excluded relevant experts from some non-English-speaking countries. Nevertheless, several panellists were based in non-English-speaking European contexts, and the panel included representation from countries where SP is currently implemented across Europe. Recruitment through professional networks and targeted invitations may have favoured respondents already engaged in SP, potentially limiting representation of settings where SP is less established. Although revisions were restricted to wording intended to improve clarity, rewording can still influence responses and should be considered when interpreting changes between rounds. Furthermore, although the panel spanned 15 European countries, participation was skewed towards higher-income settings: the three most represented countries (Germany, the UK and Italy) accounted for 44% of respondents and rank among the highest-Gross Domestic Product (GDP) countries in Europe. This imbalance may limit the applicability of our findings to lower-resource or less affluent settings and, given that SP has the potential to either narrow or widen health inequalities, should be considered from an equality, diversity and inclusion perspective; SP does not operate in isolation from the socioeconomic context of the systems in which it is implemented. In addition, this study did not incorporate patient and public involvement; given that SP is inherently patient-centred and needs-led, the absence of service-user input in study design and interpretation is a further limitation that should be addressed in future consensus and implementation work. Another limitation is attrition across the Delphi rounds, which is a well-known risk with Delphi studies.[46–49]

Of the 85 panellists recruited, 75 completed Round 1 (75/85; 88%), meaning 10/85 (12%) did not participate in Round 1. Of those who completed Round 1,

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57 completed Round 2 (57/75; 76%), meaning 18/75 (24%) did not complete Round 2. All Round 2 respondents completed Round 3 (57/57; 100%). Panel attrition is a recognised challenge in Delphi studies and was anticipated in the study protocol; attrition of this magnitude is commonly reported between rounds, indicating that retention in the present study was acceptable.

CONCLUSIONS

Across three Delphi rounds, an international European panel converged on actionable, consensus-based recommendations for SPLW practice and for accessible, context-adaptable implementation of SP in primary care. Consensus was strongest for a person-centred, relational SPLW role that explores 'what matters', supports navigation to community and social care resources, advocates when barriers arise, and provides structured follow-up and documentation. Panellists also endorsed clear workforce supports—mandatory training in empathic communication and community-resource navigation, plus ongoing supervision and continuing professional development. At the system level, the panel agreed that funding and employment models must be tailored to national and local contexts, supported a cofunded multisector approach as a sustainable option, and emphasised maintaining multiple access routes (including primary care referral, self-referral and referral from secondary care with clear boundaries). Digital tools were viewed as useful complements when resources are scarce but not substitutes for trust-based link working. Several design questions remained without consensus (eg, preferred professional background, formal certification and restrictions on specific referral routes), underscoring the need for context-sensitive adaptation and comparative implementation research as SP scales across diverse European systems.

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